

FIDUCIARY DOCUMENTATION · CHECKLIST

Your Fiduciary File.

Five categories of pharmacy-benefit documentation courts evaluate when assessing prudent process. Required records, what "good" looks like, and the one-page test that reveals your gaps. Litigation does not wait for next quarter.

01 The Five Documentation Categories

Check off each item currently in your file. Anything unchecked is a gap worth closing this quarter.

PBM Selection and Procurement

Decision trail · pre-signing

What good looks like: a dated memo before signing that explains the decision and lists the alternatives considered.

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| ☐ RFP document with evaluation criteria defined before proposals | ☐ Proposals from at least three PBM candidates |
| ☐ Evaluation matrix scoring each proposal against criteria | ☐ Selection committee meeting minutes and rationale memo |
| ☐ Conflict of interest disclosures from committee members | ☐ Evidence of clinical / financial / legal expertise on the committee |

Contract Negotiation

Redline trail · not a clean signature

What good looks like: evidence you did not sign the PBM's standard contract unchanged. A clean signature page is the gap.

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| ☐ Redlined contracts showing changes from PBM's initial terms | ☐ Legal review from benefits counsel (not just general counsel) |
| ☐ Benchmark analysis comparing your terms to market | ☐ Documentation of negotiated pricing guarantees and rebate pass-through |
| ☐ Documentation of negotiated audit rights and termination provisions | ☐ Side letters and amendments with execution dates |

Ongoing Performance Monitoring

Quarterly cadence · most common gap

What good looks like: quarterly records showing someone reviewed the numbers against guarantees and acted on shortfalls.

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| ☐ Quarterly PBM performance reports against contract guarantees | ☐ Independent analysis verifying PBM-reported performance |
| ☐ Documentation of guarantee shortfalls and how they were resolved | ☐ Annual benchmarking against market comparators |
| ☐ Clinical program outcomes data and review | ☐ Committee meeting minutes reviewing the quarterly data |

Renewal and Amendment

Market check · not auto-renewal

What good looks like: evidence renewal was a decision, not automatic. Auto-renewals are the easiest plaintiff gap.

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| ☐ Market check or RFP at renewal (even if you stay with the current PBM) | ☐ Analysis comparing renewal terms to what else is available |
| ☐ Documentation of what you negotiated at renewal | ☐ Rationale memo for the renewal decision |
| ☐ Amendment tracking log with execution dates | |

Committee Governance

Cadence + minutes · documented decisions

What good looks like: regular documented meetings where pharmacy decisions are discussed, data reviewed, actions recorded.

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| ☐ Named fiduciary or committee charter for pharmacy benefit oversight | ☐ Regular meeting schedule, quarterly at minimum |
| ☐ Meeting agendas with pharmacy items called out | ☐ Meeting minutes with decisions and rationale recorded |
| ☐ Attendance lists showing consistent participation | ☐ Action items with completion tracking across meetings |

03 The One-Page Test

If a plaintiff's attorney asked you to demonstrate prudent pharmacy-benefit process today, could you produce these five documents within 24 hours? If any of the five is missing, that gap is your exposure. Start building it now. Not next quarter. Now.

DOCUMENT 1 · SELECTION

A selection rationale memo dated before contract signing.

References the evaluation criteria, names the alternatives considered, and explains why this PBM was chosen over the others. The date matters: a memo written after the fact does not demonstrate prudent process.

DOCUMENT 2 · NEGOTIATION

Evidence of contract negotiation, not just acceptance.

Redlines, side letters, market benchmark analysis, or counsel memos showing terms were tested before being signed. A clean signature page with no negotiation trail is the documentation gap plaintiffs look for first.

DOCUMENT 3 · MONITORING

Quarterly performance monitoring documentation.

Performance reports against contract guarantees with evidence someone reviewed them and acted on shortfalls. Annual reports without quarterly review are insufficient; the standard is ongoing prudent oversight.

DOCUMENT 4 · RENEWAL

A renewal evaluation showing alternatives were considered.

Market check, renewal benchmarking analysis, or rationale memo demonstrating renewal was a decision, not an automatic continuation. Apply this even if you stay with the incumbent PBM.

DOCUMENT 5 · GOVERNANCE

Committee meeting minutes with pharmacy-specific decisions recorded.

A consistent quarterly cadence of documented committee meetings where pharmacy data is reviewed, decisions are made, and actions are tracked to completion. Ad hoc and undocumented decision-making is the highest-risk pattern.

04 Closing Documentation Gaps This Quarter

If the One-Page Test surfaces missing documents, the fix is to start building forward, not to manufacture backdated records. Three steps to take this quarter, in this order:

§ Audit your existing documentation trail

Pull the file. Map what exists against the five categories above. Catalog the gaps in writing. The audit itself is the first piece of documentation; date it, save it, and reference it in committee minutes.

§ Establish a documentation protocol going forward

Define what gets created at each touch point with the PBM: every renewal, every amendment, every quarterly review. Assign an owner. Decide where files live. The protocol exists once; the files compound from there.

§ Engage benefits counsel for a fiduciary file review

Once the audit is complete, share it with qualified ERISA counsel. They will identify priority remediation items based on current case law. The cost of this review is materially less than the cost of being unprepared in litigation.

§ Document the new cadence in committee minutes

Update the committee charter to reflect the documentation protocol and quarterly review cadence. Distribute, get sign-off, and reference the updated charter in the next meeting minutes. The charter is itself fiduciary evidence.